The Unspoken Curriculum

How Women Navigate Systemic Healthcare Failures and Forge Connection Through Lived Experience

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EXECUTIVE SUMMARY

- Across 45 participants from diverse backgrounds, this report reveals how women are reframing medical dismissal as a systemic failure rather than a personal flaw.
- Significant gaps in medical education on women's health create fragmented care, forcing patients to navigate a confusing and uncoordinated system on their own.
- In response, women are cultivating strong self-advocacy skills, learning to trust their inner voice and negotiate for the care they deserve.
- Ultimately, peer storytelling emerges as a powerful lifeline, providing the validation and connection needed to counteract isolation and build collective resilience.

PARTICIPANT DEMOGRAPHICS

45	24–65	17	30
PARTICIPANTS	AGE RANGE	ETHNIC BACKGROUNDS	REGIONS

AGE DISTRIBUTION

18-24	1
25-34	6
35-44	22
45-54	9
55-64	6
65+	1

Mean: 43.1 | Median: 43

ETHNICITY

Caucasian	19 (42.2%)
WOC	8 (17.8%)
Indian	3 (6.7%)
Mexican	2 (4.4%)
South Africa	1 (2.2%)
Canadian	1 (2.2%)
Japanese	1 (2.2%)
Carribean	1 (2.2%)
Israeli	1 (2.2%)
Afghani	1 (2.2%)
Filipino	1 (2.2%)
Latino	1 (2.2%)
Australian	1 (2.2%)
Trinidadian	1 (2.2%)
Pakistani	1 (2.2%)
Chinese	1 (2.2%)
Nigerian	1 (2.2%)

GEOGRAPHIC SCOPE

33	12
US-Based	International

PA	4
CT	3
CA	3
MA	3
Canada	2
NY	2
AZ	2
NJ	2

+ 22 more

KEY INSIGHTS

When faced with a healthcare system that dismisses their symptoms or labels them as emotional, women are finding empowerment by reframing the experience not as a personal failing (e.g., being 'crazy' or 'dramatic'), but as evidence of a broken system, which shifts the focus from self-blame to seeking validation and advocating for change.

> *"Your moods and your hormonal chaos is not a character flaw. It's a biological sign. So first of all, I always say this, you're not crazy, you're not dramatic, and you're not overreacting, right? You are just overdue for supports and overdue to be seen as to be understood."*
>
> — PATIENT • EP. UTS_S4_PODCAST_042.DOCX

> *"it's no need to fix what's not broken because you are not broken. The system is so, healing is more about being seen."*
>
> — PATIENT • EP. UTS_S4_PODCAST_042.DOCX

Gaps in medical education, particularly around neurodivergence, mental health, and the experiences of marginalized communities, result in clinicians misinterpreting symptoms, leading to incorrect diagnoses (e.g., anxiety/depression instead of ADHD) and prolonged patient suffering.

> *"I've met so many women that were on anxiety medication or depression medication for decades, and it never got better. And then lo and behold, they discover that, oh wait. It wasn't actually anxiety and depression at all, or underlay at all, was a DHD. And so when you treat the A DHD, the anxiety and depression also resolve, or at a minimum gets so much better."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_010.DOCX

> *"it's not lost on me that whenever we're talking about the medical industrial complex, we are also talking about black people whose bodies were used without anesthesia, whose bodies were used without their consent, right? I think of people like Henrietta La I think of all of the black bodies that were experimented on."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_014.DOCX

Significant gaps in medical education and training, particularly around hormonal health, menopause, and survivorship care, result in fragmented and inadequate support for women. Patients find their symptoms are often misunderstood or misattributed, such as hormonal issues being treated solely as mental health conditions, forcing them to navigate a system that lacks a holistic understanding of their health.

> *"I was putting together puzzle pieces, but not just puzzle pieces. It was almost like someone had dumped a box of puzzle pieces. On me. I didn't have the top of what that puzzle looked like, and I didn't know what was survivorship, what was menopause, what was just, you know, uh, mourning the future that I thought that I was gonna have."*
>
> — PATIENT • EP. UTS_S4_PODCAST_026.DOCX

> *"I believe that psychologists and psychiatrists should be educated in hormones because they will be a much better doctors for their patients."*
>
> — PATIENT • EP. UTS_S4_PODCAST_028.DOCX

Clinicians and patients express frustration with a healthcare system characterized by inconsistent standards of care across different regions and specialties, stemming from historical gaps in women's health research and medical training.

> *"I find it unique that people op that we operate differently across state lines. It could really help if we took what this state was doing well and what this state was doing well, and we just did that, it will really, really, really help."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_037.DOCX

> *"We have a systemic issue. You know, women's health isn't understood. We've been treated like we're men. I mean, it wasn't until until 1993 that they started doing research on women conducting, bringing them into clinical trials and for a host of reasons."*
>
> — HOST • EP. UTS_S4_PODCAST_038.DOCX

The mental healthcare system operates in silos, with significant fragmentation between government ministries, public and private sectors, and the understanding of mental versus physical health, which complicates navigation for patients and prevents a holistic approach to care.

> *"there is a lot of, uh, fragmentation, even at the government level. Trauma impacts everything, not just the Ministry of Health. Right. It impacts the Minister of Welfare and Social Affairs, minister of Economy, uh, minister of Defense, obviously with all the soldiers. So I'd love to find ways to have more effective public-private partnerships on some of these."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_016.DOCX

> *"I wish more people would realize why mental health is so important. You know, it's, we're not doing mental health for the sake of mental health, although it would be worthy anyways, but it's about. Physical health. We have so much high blood pressure and cardiovascular issues in general autoimmune diseases, you know, shingles is all around here right now."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_016.DOCX

Complex, multifactorial health issues common to women, such as brain fog or postpartum mental health, are poorly managed by a fragmented healthcare system, forcing patients to navigate siloed specialties and leaving clinicians without clear, integrated treatment protocols.

> *"It's not like getting a list. Of mental health providers that you now need to call and find out if they're taking your patients, because nobody who's struggling with mental health is going to make that five calls, wait for it, and then go back."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_040.DOCX

> *"So brain fog, I would say, is one of the hardest things to nail. Like I said, hormone therapy can be helpful, but it often requires us to think a little harder about our nervous system and look a little harder sometimes at what's going on in our life"*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_039.DOCX

The healthcare system's siloed nature and lack of integrated support between appointments forces patients and their advocates to become navigators of a complex, often uncoordinated, landscape, leading to frustration and gaps in care.

> *"I find it unique that people op that we operate differently across state lines. It could really help if we took what this state was doing well and what this state was doing well, and we just did that, it will really, really, really help."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_037.DOCX

> *"And, and, and there's really no one in between appointments helping you. You gotta do it all by yourself, right?"*
>
> — EDUCATOR • EP. UTS_S4_PODCAST_035.DOCX

The healthcare system's fragmentation and lack of accessible, centralized information create significant navigation difficulties for patients, particularly those with complex needs or in post-treatment phases like cancer survivorship. Patients describe their search for knowledgeable and coordinated care as an 'odyssey,' where they are left to connect the dots between disparate specialists and resources on their own.

> *"I was released into a world that I didn't quite know how to navigate, and I felt like. With the menopausal symptoms that I was experiencing, my lack of, of knowledge of what those look like. I was putting together puzzle pieces"*
>
> — PATIENT • EP. UTS_S4_PODCAST_026.DOCX

> *"finding a doctor that had the knowledge to treat me, that was an odyssey, to be honest"*
>
> — PATIENT • EP. UTS_S4_PODCAST_028.DOCX

Clinicians recognize and experience the frustrations of a fragmented healthcare system, characterized by access barriers and navigation difficulties. In response, they take on the role of advocates for their patients, fighting insurance denials and helping them find appropriate care, while also developing personal coping strategies to manage the stress of working within a broken system.

> *"I advocate for my patients, you know, when we're running against, you know, insurance is not covering this medication or insurance is not willing to. Provide, um, additional days in the hospital when we know that this person needs it."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_020.DOCX

> *"And so for me personally, it's about being that guiding light. However, I can be using my platform as an O-B-G-Y-N and you know, having a national role with ACOG to be able to shine that light for other moms."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_019.DOCX

Clinicians, particularly those who are first-generation doctors, experience significant burnout and isolation during their medical training due to a lack of mentorship, financial resources, and formal education on self-care and success beyond clinical skills. This training gap creates a cycle of unwellness where providers struggle to care for themselves, let alone their patients.

> *"I think by the time I finished medical school, I was already burned out. Like life had been hitting me up inside, you know, I was already burned out. Uh, but no one had, no one told me what success looked like, right? I hadn't seen what success looked like... And if I had a mentor, maybe I would've learned some of those things. But unfortunately, there were no mentors."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_001.DOCX

> *"'cause you do learn in school. There are some practitioners who are very, you know, strictly psychoanalytic and we're a mirror and we don't show any emotion or whatever, but. You know, I feel like if you're on the ride with me, um, you know, for some people I am a sister, you know, I'm playing a role of a sister."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_003.DOCX

Patients and advocates are actively challenging a healthcare system that often applies a standardized, male-centric approach to diagnosis and treatment. They recognize the need for different therapeutic approaches for women and diverse populations, and are forced to engage in persistent self-advocacy—questioning diagnoses, demanding specific tests, and fighting insurance denials—to receive care that acknowledges their unique physiology and circumstances.

> *"You cannot just give me the pet idea or pet uh, diagnosis, which might only work for a man or for a child."*
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> — ADVOCATE • EP. UTS_S4_PODCAST_032.DOCX

> *"I go every year. I don't care about the insurance companies, whether we have to pay cash, whatever it is. This is a very deadly disease."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_033.DOCX

A pervasive culture of silence surrounding women's health challenges, particularly during postpartum and midlife transitions, fosters profound feelings of isolation, shame, and the mistaken belief that one is alone in their struggle.

> *"So again, there, there's just so many ways to be. Isolated, like in, in postpartum. Um, and I, I'm very passionate about it, so that's why I wanna talk more about that aspect. Uh, even with breastfeeding, there's so much shame and judgment with it."*
>
> — PATIENT • EP. UTS_S4_PODCAST_040.DOCX

> *"You're connecting the dots that this is not me alone in my journey. This is what women in general are going through."*
>
> — HOST • EP. UTS_S4_PODCAST_042.DOCX

A pervasive culture of shame and stigma around topics like trauma, domestic violence, and mental health challenges forces individuals into isolation and silence, preventing them from seeking help and perpetuating cycles of suffering.

> *"as you were describing the whole domestic violence thing and the fact that there's so much shame attached to it and it's not talked about, I think the greatest thing we could do in this community."*
>
> — HOST • EP. UTS_S4_PODCAST_015.DOCX

> *"when you have individual trauma, there's still a bit stigma, but today there's no more stigma. We all got hurt and I mean, of course, like we're still working with people, but the number of people who told me, you know, no one knows I have PTSD from the war like 20 years ago, and now it's. Like you could talk about it."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_016.DOCX

Women often feel isolated by societal expectations and stigma, particularly around life choices like marriage and children, or when navigating mental health challenges. This isolation is a significant barrier to wellbeing, prompting the creation of hybrid (online and in-person) communities designed to meet people where they are and foster connection without judgment.

> *"I think a lot of women feel alone in this journey because they're not really understood, not understood at home, not understood in the academic systems. At a corporate level, it's very isolating."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_005.DOCX

> *"if I had a dollar for every time someone said, are you married? Do you have kids within the first minute of meeting me, um, I would be very, very rich. And so I'd like to shift that."*
>
> — PATIENT • EP. UTS_S4_PODCAST_009.DOCX

Individuals, particularly professionals like doctors and athletes, risk losing their sense of self when their identity becomes too tightly bound to their professional role or title. This can lead to feeling 'invisible' or lost without the external validation of that role, highlighting the need to cultivate a multifaceted identity and sense of contribution beyond one's career.

> *"I took out that white coat and no one recognized who I was. I would walk to professors who I had spent time with that same day, and they didn't know me by name. And I thought, wow, that's so odd that a coat could change how people see you and you take that coat off and then you're invisible again."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_001.DOCX

> *"yes, you're this athlete brilliant, but you are also so much more than that. And actually getting them to work with that and, and getting them to think about all those other things and areas that they're of interest and that they love to do"*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_004.DOCX

Women, particularly in professional or high-achieving contexts, often feel an intense internal and external pressure to endure struggles in silence, believing they 'should be working harder' or 'doing more' rather than giving themselves permission to rest, seek joy, or acknowledge their mental health needs.

> *"especially with women, we, we feel like, well, no I shouldn't, I should be working harder. I should be doing more. I should, no, I shouldn't go meet my friend from margaritas. I should really be mopping the floor. I should, the floor will be there tomorrow. It'll be okay."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_018.DOCX

> *"I'm a psychiatrist and this is, this is what I do and I have to do it this way, where a younger version of me might've said, might've come to the conclusion I came to a long time ago."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_018.DOCX

Faced with provider dismissal, misdiagnosis, and a lack of validation, patients are forced to reclaim their agency by trusting their own bodily intuition and becoming the ultimate experts on their own health, often after enduring years of ineffective or harmful treatment.

> *"you are the expert on you do not abdicate your mental health to anyone. Doctors do not know more than you about what works for you, and the simple reason, I mean, they may know all the medical stuff, right. But bottom line, you are the only one that has that rudder inside yourself."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_010.DOCX

> *"sometimes we are told, especially as women, um, when we go to the doctor's office, it's all in your head. Well, your head, as you said, is attached to our body. And so it, you know, and there are all sorts of physiological connections as we've talked about. So, um, you know, own that and, and say, okay, so then ad address it."*
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> — ADVOCATE • EP. UTS_S4_PODCAST_013.DOCX

When patients experience dismissal or a lack of answers from the healthcare system, they are compelled to take on the role of researcher and advocate for their own health. This often involves meticulously tracking symptoms, educating themselves on complex medical topics, and learning to persistently question providers and change doctors to overcome diagnostic delays and receive necessary care.

> *"After when you've been for three years and nobody's helping you. You need to do your own research. You need to advocate and learn and educate yourself."*
>
> — PATIENT • EP. UTS_S4_PODCAST_028.DOCX

> *"so many of the women that I work with, that I know that I interviewed, they have been dismissed by their healthcare providers. Oh, you're just postpartum. What kind of answer is that"*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_027.DOCX

Patients are encouraged by clinicians to be active participants in their own care by asking questions, seeking information, and advocating for their needs, treating therapy and medicine with the same level of inquiry. If a provider is unwilling to engage in this collaborative conversation or makes the patient feel dismissed, it is a clear signal to seek care elsewhere.

> *"I mean, you should be an advocate for yourself in therapy just like you hopefully are in medicine. It's really important. It's appropriate, and your doctor or provider should be more than willing to answer your questions, and if they're not, then maybe you need to seek treatment elsewhere."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_002.DOCX

In a healthcare system where dismissal is common, patients must learn to fiercely advocate for themselves, trust their own judgment, and be willing to change providers, framing self-advocacy not just as a preference but as a crucial act of self-preservation.

> *"you are worth fighting for If there's ever an environment space provider that leaves you feeling like you're not. Move swiftly and pivot, you are worth fighting for."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_014.DOCX

> *"sometimes our choices in life, the options are shitty and shittier. Right, but we're still making a choice and there there's power in that. So that's something that I personally try to remind myself and practice, and also help the clients that I work with focus on as well. You get to be in charge."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_015.DOCX

In response to a system that may not adequately listen or respond, women are developing strong self-advocacy by learning to trust their inner voice, treating their feelings as valid biological signals, and actively negotiating for the care and life they deserve.

> *"Listen to yourself... eventually that leaves the space to hear your inner voice and then follow it. That's what I would say."*
>
> — PATIENT • EP. UTS_S4_PODCAST_043.DOCX

> *"how do you negotiate with yourself so that you really advocate for what, what. You deserve. I think that's really, and, and how you wanna set up your life and operation."*
>
> — PATIENT • EP. UTS_S4_PODCAST_041.DOCX

For clinicians and others in caregiving roles, the risk of burnout and vicarious trauma is immense, making the intentional practice of setting boundaries—such as unplugging from work and pouring back into oneself—a non-negotiable requirement for sustained effectiveness and personal wellbeing.

> *"I think the hallmark of boundaries in this work is. In order to be effective at what I do as a provider, I ha I cannot take it on. I have to purposefully unin mesh, right? I have to really be diligent and vigilant about. Unplugging and making sure that I can pour into me so that I can show up as the container that I want to be for my patients."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_014.DOCX

> *"I think being in private practice can be very isolating and lonely. And you also wanna be able to check in like, am I on the right track with where I'm going with this client? Or I'm really stuck with this client. Do you have any ideas? It, it can be a really important piece of the work."*
>
> — HOST • EP. UTS_S4_PODCAST_015.DOCX

Women, especially those in caregiving roles, are increasingly recognizing that prioritizing their own mental and physical health is not selfish but a necessary foundation for creating stronger families and communities.

> *"On planes, they say, put your mask on first, and we've been doing it wrong this entire time. We take care. We deliver these babies, we get married, and we take care of everybody but ourselves. But we have to put our mask on first."*
>
> — EDUCATOR • EP. UTS_S4_PODCAST_037.DOCX

> *"I think one of the things I would say that I'm doing for, for marching. For my own mental wellness within all of this is to make space to take care of myself through this and learning to be able to ask for help and learning to make space for myself and model that."*
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> — ADVOCATE • EP. UTS_S4_PODCAST_036.DOCX

Clinicians are actively working to destigmatize mental health by reframing emotional struggles as universal human experiences rather than permanent, defining illnesses, especially for young people. They achieve this by using accessible language (e.g., 'sadness' instead of 'depression'), sharing their own vulnerabilities, and focusing on skill-building for emotional management to foster resilience.

> *"I don't talk about depression with the young ones. I'll say sadness and, and nervousness or fear anxiousness. To try to, once again, you know, kind of normalize the experience of emotions."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_002.DOCX

> *"I've had teenagers say, because I'm mentally ill. So a lot of my practice has been normalizing this experience that the emotions might be really strong, but we could bridle them, you know, and it doesn't mean you are a mentally ill person."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_002.DOCX

Sharing personal stories and listening to the lived experiences of other women serves as a powerful antidote to isolation, providing the validation, connection, and sense of shared humanity needed to navigate health challenges and systemic failures.

> *"healing is more about being seen. So what you need is find a place that you know you can feel validated. Connect with other women. You know, listen to other women's stories and don't stay stuck"*
>
> — PATIENT • EP. UTS_S4_PODCAST_042.DOCX

> *"why are we, if, if half of our population is having this, why don't we ever talk about it? Why are we so isolated and why do we struggle in silence?"*
>
> — PATIENT • EP. UTS_S4_PODCAST_040.DOCX

Individuals experiencing social isolation and lack of support find profound relief, empowerment, and a renewed sense of self-worth by actively seeking and creating peer communities where they can share their lived experiences without judgment.

> *"you definitely need to. Have community and be open to sharing your experience because you'd be surprised how many people are going through the same thing at the same time. And I learned that going forward. Like, yeah, I'm just going to speak and then, you know, if I, if I can help somebody or if you know someone can help me, that's great."*
>
> — PATIENT • EP. UTS_S4_PODCAST_011.DOCX

> *"we know that we are doing it for each other and doing it very safely so that each one of us can get up at the end of that conversation, carry on with our lives, but with a sense of empowerment again, and feeling that somebody has my back."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_012.DOCX

Individuals overcome feelings of social isolation and lack of support by actively seeking out peer communities where they can share their personal stories. This act of sharing and being witnessed provides profound validation and connection, reminding them they are not alone in their struggles and empowering them to move forward.

> *"at the beginning I thought that maybe I was unlucky. But then it was like every woman that I was meeting telling me, sharing me their horror stories and saying like, with frustration, it's like, but where can I find information?"*
>
> — PATIENT • EP. UTS_S4_PODCAST_028.DOCX

> *"I really believe in the power of women coming together in sisterhood."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_027.DOCX

In the face of both individual and collective trauma, actively building and leaning into a peer community is a primary protective factor, fostering a sense of belonging and shared strength that enables collective healing.

> *"collective trauma leads to collective healing. Which in a way I like that. I like that. I like that a lot. That's good. We look at each other here and we're all in it. And sometimes we forget, but most of the time when you slow down, you realize we're all in it together and we are gonna heal together. We got hurt together, we're gonna heal together."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_016.DOCX

> *"Lean into your community. Lean into your community. Lean into your community. You know, when this first started the case, many cultures, I went back to the elders, you know, my, my aunts and uncles. What am I supposed to do about this? What's supposed to happen? They've seen these types of things, right? They were in the sixties and they've seen these types of things, and everyone said, community, we rely on each other."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_018.DOCX

Individuals experiencing a lack of support, particularly during challenging life or health transitions, actively seek out and create peer communities. Sharing personal stories within these safe spaces fosters a sense of connection and validation, which counteracts the profound isolation that can exacerbate health challenges.

> *"don't give up on sharing that. 'cause you're going to find your tribe, you're going to find your audience that allows you to help another person and allows you to continue to grow."*
>
> — PATIENT • EP. UTS_S4_PODCAST_046.DOCX

> *"It's the isolation that brings them further and further away."*
>
> — HOST • EP. UTS_S4_PODCAST_045.DOCX

In a society where individuals and organizations often operate in silos, intentionally building community and coalitions provides a critical defense against social isolation, creating the support systems necessary for personal resilience and sustained collective action.

> *"we're building community because that's one of the things that's really missing. From a lot of people's lives in our society, we've forgotten how to rely on each other and how to work with each other."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_036.DOCX

> *"So it, it has been a community journey, which I'm very grateful for."*
>
> — PATIENT • EP. UTS_S4_PODCAST_038.DOCX

The fear of judgment and societal stigma often leads individuals to suffer in silence and social isolation. However, the act of speaking up and sharing personal stories, especially about sensitive topics like mental health, serves as a powerful antidote. This vulnerability fosters connection, reduces shame, and creates a virtuous cycle where openness encourages others to share, collectively breaking down stigma.

> *"It's only been recently that I started talking openly about my eating disorder. I felt very embarrassed by that and also nervous about. My kids knowing about that for some reason"*
>
> — PATIENT • EP. UTS_S4_PODCAST_031.DOCX

> *"Open up a little bit to people who you feel like you can see them, anything, and they will not judge you, you know?"*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_032.DOCX

Sharing personal stories through media platforms acts as a powerful 'release valve' for the storyteller and breaks down the barrier of silence around difficult experiences. This public act of vulnerability fosters a sense of universal human connection, validating the experiences of both the speaker and the audience, and demonstrating that every individual's story has the power to touch and impact others.

> *"as you begin to give voice and tell your story, there's something that's like a release valve, if you will, there."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_032.DOCX

> *"I've never really shared my story with someone, but I'm honored to share it today with you."*
>
> — PATIENT • EP. UTS_S4_PODCAST_033.DOCX

The most effective way to combat the systemic silence, stigma, and educational gaps surrounding women's health issues like menopause is through open conversation and the sharing of personal stories. When women communicate their experiences within a community, it creates awareness, normalizes the conversation, and empowers others to seek information and support, ultimately driving a collective demand for better solutions.

> *"Thing that can change this gap is talking about it. So from a marketing background, I will say marketing and branding, that's not how it's position in the article, but it's about talking about it. And I think going back to telling stories and going back to the work you do and the community that, and how women communicate for ages and ages and uh, through time, our stories, this is what will help us put. An eye on the problem so we can solve it."*
>
> — PATIENT • EP. UTS_S4_PODCAST_038.DOCX

> *"if you don't talk about it, when you start to talk about the reality of what we're all facing, what we're all dealing with, there's a shift there that can happen just by like getting it out there."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_036.DOCX

Clinicians find that integrating their personal lived experiences, such as being a parent or managing their own anxiety, into their practice deepens their empathy and builds stronger, more trusting relationships with patients. This contrasts with traditional training that often encourages emotional distance, highlighting that personal vulnerability can be a clinical asset.

> *"But being a parent and helping parents, it is a whole different experience. It is so nice that I could say to them, you know, I, I, I get it, and I, I'm telling you to do something and sometimes I forget to do it. So it's, it's, you know, I've been there. I know, I understand. It's hard."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_002.DOCX

> *"The people who are privileged enough and who go into that. Space to sort of help other people often to have the empathy and compassion you have to live through some of it."*
>
> — HOST • EP. UTS_S4_PODCAST_001.DOCX

Individuals who have navigated significant personal challenges, such as a cancer diagnosis or other life traumas, often find purpose and healing by creating peer support systems for others on similar journeys. These initiatives, built on lived experience, provide a unique form of validation and connection that formal systems may lack, demonstrating that shared stories are a powerful resource for community building.

> *"some women don't have the stories. They don't have moms and sisters and aunts that they can ask about what happened to you at this age? I'm experiencing this. What about you? Amazing that we have a platform where we can tap into a resource and listen to lived experience because it's so important."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_005.DOCX

> *"Purpose and self-worth. And when you can see that by your interaction with another person. ... Oh, I'm valuable. I am valuable to this person in this moment. And, and that, that stores, right, that's, that's one that's a layer in the brain."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_009.DOCX

A woman's personal health advocacy is often sparked by witnessing the health struggles and information gaps faced by the previous generation, particularly her mother, motivating her to break cycles of silence and build better solutions for the future.

> *"I always wanted to do something in health and wellness, especially around that late thirties, right? Because of my mom and because of what I was going through, I wanted to do something."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_041.DOCX

> *"just to create a balance so that my lived experience does not put my work in a box because I lived experience has a lot of limitations... I'm open to listening to other women's stories."*
>
> — EDUCATOR • EP. UTS_S4_PODCAST_042.DOCX

Women actively challenge societal stigmas and prescribed life paths (e.g., marriage, motherhood, neurotypicality) by speaking up, sharing their stories, and defining their own measures of a joyful, purposeful life.

"I would love for society to stop asking those questions of women. Um, and I think we're getting closer. We're warming up to that. Uh, but that's not where, even if we do those two things amazing, that's not what our value is, right? We bring so much as every human brings so much as a part of our human, um, to the conversation, to the table. And so let's not lead with are you married, do you have kids as the two most important things that a woman can do with her time."

— ADVOCATE • EP. UTS_S4_PODCAST_009.DOCX

"I just would invite women and humans just to live in your true self, um, and to show up. And gosh, life is so short. If there's one thing my experience with my dad taught me is that. There is no death does not discriminate new slash we're all gonna die and we don't know when. And, um, oh, if we could just show up every day living and doing things that bring us joy in the way that we care about, um, and not for what other people think that would be, that would be my mess message to women and to, and to all humans."

— ADVOCATE • EP. UTS_S4_PODCAST_009.DOCX

Women, often encouraged by clinicians and advocates, find immense validation, connection, and hope by seeking support within a peer community and sharing their personal stories. This process combats isolation, normalizes their experiences, and empowers them to navigate health and life challenges.

"And it took so many other people telling me their stories and being like, me too, and let me show you the way."

— CLINICIAN • EP. UTS_S4_PODCAST_019.DOCX

"Whatever you're feeling and experiencing, whatever your circumstances may be, you are not alone. Okay? Even if there's no one around you, trust, you're not alone."

— CLINICIAN • EP. UTS_S4_PODCAST_018.DOCX

Individuals, particularly post-COVID, experience profound loneliness and social isolation due to the loss of 'third spaces' and supportive networks. Clinicians and advocates identify that fostering human connection—whether through therapy, peer groups, or sisterhood—is a critical component of mental wellness, where sharing burdens divides sadness and sharing joys multiplies them.

> *"A lot of people are lonely, especially post COVID. People are lonely. People have lost, um, a lot of the, we call them third spaces, right? We used to have work home and then some other space that you went that had nothing to do with your job or your family, but it was someplace you felt connected"*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_003.DOCX

> *"sadness shared is divided and joy shared is multiplied. That's what it is. ... We share the burden and we also celebrate the victories and we can care for each other, right?"*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_003.DOCX

Patients face significant, systemic barriers to accessing consistent and affordable healthcare, driven by factors like high-deductible plans for low-income individuals, provider shortages due to low reimbursement rates, and insurance plan changes that disrupt established patient-provider relationships. These issues create a fragmented and frustrating care experience, particularly for those managing chronic conditions or life-stage transitions like menopause.

> *"you'd put somebody in a high deductible healthcare plan. Who's making $40,000 a year who basically can't seek care and didn't have salary to defer. And so some of these things just felt so broken."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_046.DOCX

> *"if my employer changes a carrier every year and I have to change physicians, it's really hard for me to seek the right type of mental health care...maybe I've just gotten all my menopause on track and now I've gotta switch physicians."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_046.DOCX

When workplace structures and values do not align with the needs of working mothers, women are often forced to leave their jobs to find or create roles that offer the flexibility required to be present for their families and protect their own mental health.

> *"My previous job in Atlanta there, there wasn't a type of position like that. And you know, I tried to see if they would create that for me. And they, they couldn't, that wasn't what they needed. Value's not aligned. And so I left and I became a hospitalist"*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_019.DOCX

> *"I used to think it was really cool. You know how I'd be like, moms kind of brag, oh, I'm just so busy. I have so much going on. And that's kind of a badge of honor. I used to wear that badge of honor. ... and now I, I don't do that. It, there's nothing to brag about there. It's kind of sad."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_021.DOCX

Fear of Stigma and Discrimination Creates a Culture of Health Secrecy in the Workplace

> *"I would say there's a majority of employers that are not safe spaces for them to know your healthcare outcomes. And if those leaders know you're seeking, you're currently accessing care for cancer. Maybe you're not up for promotion. If they know you're currently accessing in vitro services, they know you're planning for a baby."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_046.DOCX

> *"it's no coincidence, right? Because you get to an age where you're ready to advance in leadership and you're confronted with these transitional symptoms that menopause and, and other things can bring."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_045.DOCX

The immense, often invisible, labor of caregiving within families is a significant source of strain for women, a burden that is frequently compounded by the silent grief of family estrangement or loss.

> *"caregiving is love in action, but for women it's often invisible labor. And so we want to be seen. Everyone wants to be seen. You wanna have that feeling, but caregiving is an invisible labor that you're not getting any rewards for."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_044.DOCX

> *"I meet so many people now who are estranged from one or more of their family members and how that, you know, impacts them for bad or good."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_045/DOCX

When a career that has defined one's identity ends or shifts dramatically, it can precipitate a period of profound grief, confusion, and loss of self-worth. This painful emotional journey forces a decoupling of identity from professional achievement, creating space for a reinvention of self that is guided by inner curiosity and purpose rather than external validation.

> *"I can't root my identity in my accomplishments. Because when your identity is rooted in your accomplishments, it's like you're always having to hustle for your worthiness."*
>
> — PATIENT • EP. UTS_S4_PODCAST_030.DOCX

> *"What am I here to do with my life? Because I actually don't know."*
>
> — PATIENT • EP. UTS_S4_PODCAST_034.DOCX

A growing number of professionals are actively rejecting the 'hustle culture' by establishing clear boundaries that place their personal and family well-being above work demands. This behavioral shift involves a conscious decision to prioritize self-care and family, with the understanding that a job is a part of life, not the entirety of one's identity or worth.

> *"we are number one, family is number two, and and unicorn is number three. And we all have to ensure that we are adjusting that way. And our team knows this as well. And anytime there is a moment we're like, no, you are first. Nope. Your family like, focus there. The job will get done."*
>
> — PATIENT • EP. UTS_S4_PODCAST_023.DOCX

Women are actively pushing back against the expectation to be constant caregivers by learning to set boundaries, say 'no,' and prioritize their own needs within the family unit. This involves a conscious mindset shift to take time for themselves without feeling guilty, recognizing that their well-being is as important as that of those they care for.

> *"we're too busy trying to take care of, and I speak from experience everyone else around us, but why we are just as important as women and we deserve that love and we, we do whatever you see you need to do to take care of yourself. Going on walks, you know, reading a book, taking that five minutes for yourself and not feeling guilty. Setting boundaries. Saying no stuff can wait."*
>
> — PATIENT • EP. UTS_S4_PODCAST_024.DOCX

Women are consciously rejecting the pressure to constantly 'do more' by actively setting boundaries and making decisions that prioritize their own mental health and joy. This behavioral shift involves giving themselves permission to rest, choose joyful activities over obligations, and protect their time and energy, recognizing that their well-being is essential, not selfish.

> *"no, I shouldn't go meet my friend from margaritas. I should really be mopping the floor. I should, the floor will be there tomorrow. It'll be okay."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_018.DOCX

> *"I recognize that I, I can't be useful and helpful, um, if I, if I'm falling apart."*
>
> — CLINICIAN • EP. UTS_S4_PODCAST_020.DOCX

The process of engaging in mental healthcare is a transformative journey of unlearning deeply ingrained societal narratives, such as the need to 'hustle for your worthiness.' Through therapy and self-reflection, individuals learn to detach their identity from external accomplishments, embrace concepts like radical acceptance, and cultivate a more stable, internal sense of self-worth.

> *"when your identity is rooted in your accomplishments, it's like you're always having to hustle for your worthiness."*
>
> — PATIENT • EP. UTS_S4_PODCAST_030.DOCX

> *"I'm very open about being in therapy and talking about a discovery I may have made about myself or a tendency I have when I was in a therapy session, you know, with my close people, but I don't hide it, and I encourage people."*
>
> — PATIENT • EP. UTS_S4_PODCAST_031.DOCX

Women find that openly sharing their struggles and realities counteracts the societal pressure to appear perfect, creating a powerful shift that dismantles stigma, fosters genuine connection, and enables collective healing.

> *"if you don't talk about it, when you start to talk about the reality of what we're all facing, what we're all dealing with, there's a shift there that can happen just by like getting it out there."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_036.DOCX

> *"Too many activist spaces where they just focus on the work and they don't ever tend to their hearts and their, their spirits. You know, you're gonna burn out this work."*
>
> — ADVOCATE • EP. UTS_S4_PODCAST_036.DOCX

There is a significant gap in postpartum care and technology that clinicians and innovators with direct, personal experience of its failings are uniquely positioned to fill, yet they often doubt their qualifications and must be encouraged to see their lived experience as essential expertise.

> *"We need people like you. In the industry like this knowledge that you have, this clinical, both personal experience and clinical experience, you know the problem, you can design something amazing."*
>
> — EDUCATOR • EP. UTS_S4_PODCAST_019.DOCX

> *"And with my second baby, I talked to a lactation consultant. I'm like, okay, here's the deal. Now I've graduated. What did I do wrong? And she's like, Andrea, of course you had low milk supply. You were exhausted. ... sometimes rest is the best thing you can do. That really made all that one little nugget, man. If I could tell everybody that one."*
>
> — PATIENT • EP. UTS_S4_PODCAST_019.DOCX

METHODOLOGY

45	179	Insights were promoted to 'High' or 'Medium' confidence based on multi-episode evidence, pattern coherence, and corroboration across speaker roles. No 'Emerging' insights were included in this report.
EPISODES	MOMENTS	CONFIDENCE